UK PRIMARY CARE · THE INVISIBLE QUEUE

The most expensive envelope in the NHS

A hospital sends one letter. It lands in a GP practice and costs far more than the stamp.

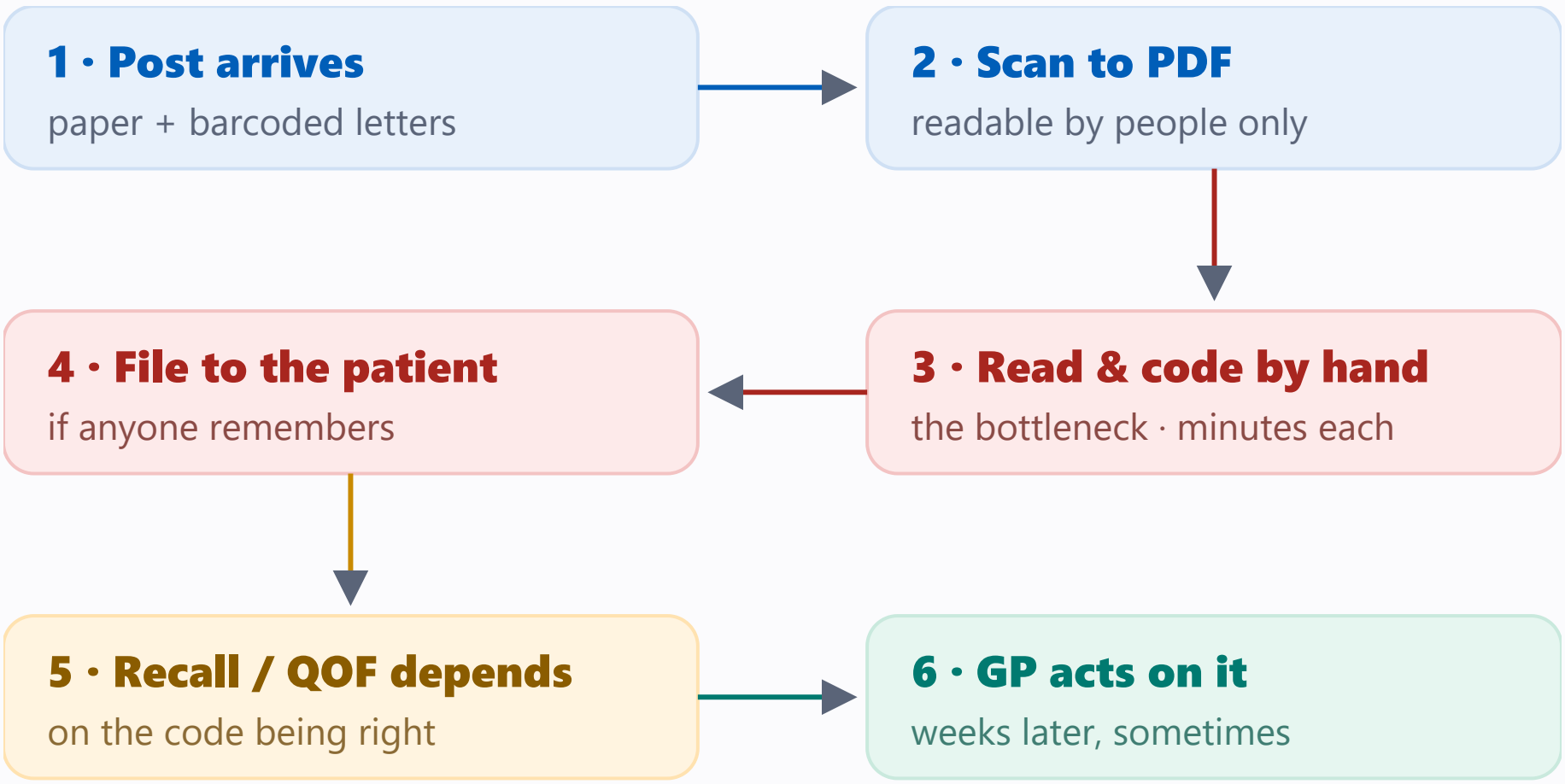
THE PROBLEM · STEP ONE

Every week, hundreds of letters arrive.

Discharge summaries. Clinic letters. Outpatient results. Each one names a patient, a diagnosis and an action — in prose, in a PDF, usually with a barcode.

THE PROBLEM · WHERE IT LEAKS

Six hands. Four chances to lose the meaning.



THE PROBLEM · WHAT IT COSTS

Three bills nobody sends.

- **Admin time**

  Every letter is read by a human before it becomes data — and read again if the code was wrong.

- **Coding gaps**

  An uncoded letter is invisible to recall, to QOF, and to the next clinician who sees the patient.

- **Delay**

  A result that sits in an inbox is clinically the same as a result that never arrived.

WHY “WE ALREADY SCAN THEM” ISN'T THE FIX

A scanned letter is a picture of information.

It cannot be searched, counted, recalled or trended. The surgery owns the file and still does not own the fact.

THE TARGET STATE

Stop filing documents. Start filing facts.

ONE PATIENT

Everything joins on the **NHS number** — not on a name typed twice.

ONE CODE

Each fact recorded as **SNOMED CT**, the standard the NHS already requires.

ONE TRAIL

Who read it, who coded it, who changed it — and when.

THE SOLUTION

HealthBridge reads the letter, proposes the code, and asks a human.

1 · INGEST

Photo or PDF → structured fields. Discharge summary, clinic letter, lab result.

2 · PROPOSE

Diagnosis, medication and action extracted, with a SNOMED CT code and a confidence.

3 · CONFIRM

A clinician accepts, edits or rejects. Only then does it enter the record.

THE SAME ENGINE, THE WHOLE VISIT

Six steps. One thread.



A letter is one round of this loop. HealthBridge runs the whole loop, so the next visit opens with the previous one already loaded.

GUARDRAILS · AND THE GATES I HAVE NOT PASSED

What it refuses to do matters more.

HUMAN CONFIRMS

No code enters a record without a clinician. No autonomous diagnosis.

TENANT ISOLATION

One practice cannot see another's patients. Ever.

UK selling needs: DTAC · DSPT · DPIA / UK GDPR · Cyber Essentials or ISO 27001 · DCB0129 with a named Clinical Safety Officer.

none of these claimed yet

PILOT

30 days. Three numbers. Then you decide.

- Minutes per letter
  before versus after
- Codes accepted
  share a clinician accepts unchanged
- Recalls closed
  by day 30, not rolled into next month

19p1362.github.io/portfolio/healthbridge

Gonela Abhilash · CPC ·
registered pharmacist · MSc
Public Health, ARU